

DYNA TETRACYCLINE CAPSULE 250MG

MAL19921286AZ

DESCRIPTION:

CAPSULE

Capsule Colour : Yellow/Orange
Capsule Size : # 1
Granules Colour : Yellow

EACH CAPSULE CONTAINS:

Tetracycline Hydrochloride 250 mg

PHARMACODYNAMICS:

Tetracycline is a broad-spectrum bacteriostatic agent and acts by inhibiting protein synthesis by blocking the binding of aminoacyl tRNA ribosome complex. Reversible binding occurs primarily at the 30S ribosomal subunit of susceptible organisms.

PHARMACOKINETICS:

Tetracycline is incompletely and irregularly absorbed from the gastrointestinal tract (GIT). The degree of absorption is diminished by the soluble salts of divalent and trivalent metals, with which tetracycline form stable complexes and to a variable degree by milk or food. It has been recommended that tetracycline should be given before food. Sodium metaphosphate may enhance the absorption of tetracycline.

As a rule, doses of 250 to 500 mg by mouth every 6 hours produce therapeutically effective plasma concentrations of tetracycline ranging from 1 to 3 mcg per ml and 1.5 to 5 mcg per ml respectively.

In the circulation, tetracycline is bound to plasma proteins in varying degrees from 24 to 65 %. It is widely distributed throughout the body tissues and fluids. Concentration of cerebrospinal fluid (CSF) is low but may raise of the meninges are inflamed. Tetracycline appears in the breast milk where concentration may be 60 % or more of those in the plasma. It diffuses across the placenta and appears in the foetal circulation in concentrations of about 25 to 75 % of those in the maternal blood. Only small amounts appear in saliva, tears, and intra-ocular fluids. Tetracycline is retained at sites of new bone formation and recent calcification, in developing teeth, and in some injured soft tissues. The biological half-life of tetracycline has been reported to be about 8 to 10 hours. Tetracycline is excreted in the urine and in the faeces. Renal clearance is by glomerular filtration and concentrations in the urine of up to 300 mcg per ml of tetracycline may be reached 2 hours after a usual dose is taken and be maintained for about 6 to 12 hours. Up to 60 % of an intravenous dose of tetracycline and rather less of a dose by mouth is eliminated unchanged in the urine; for most tetracyclines about 40 to 70 % of a dose is excreted in the urine. Tetracycline is excreted in the bile where concentrations 5 to 20 times those in plasma can occur. Since there is some reabsorption, complete elimination is slow. Considerable quantities occur in the faeces after administration by mouth and lesser amounts after administration by injection.

INDICATIONS:

Tetracycline is an antibacterial, which is effective against a wide range of microorganism including gram-positive and gram-negative bacteria, rickettsiae, mycoplasmas and chlamydiae. It is used in the treatment of infections of the respiratory tract, genital-urinary tract, and soft tissues, and for superficial infections of acne vulgaris and acne rosacea. More specialised uses are concerned with the treatment of organisms other than common bacteria, for example brucellosis, psittacosis, lymphogranuloma venereum, Q fever, primary atypical pneumonia and typhus.

RECOMMENDED DOSE:

Adults:

Antibacterial:

250 - 500 mg every 6 hours; or 500 mg to 1 g every 12 hours.

Antibacterial (anti-acne):

500 mg to 2 g daily in divided doses initially in severe cases as adjunctive therapy. When improvement is noted (usually after 3 weeks), dosage should be reduced gradually to a maintenance dose of 125 to 500 mg daily. Adequate remission of lesions may also be possible with alternate-day or intermittent therapy.

Gonorrhoea:

500 mg every 6 hours for 5 days.

Syphilis:

500 mg every 6 hours for 15 days (early syphilis) or for 30 days (late syphilis).

Usual adult prescribing limits: up to 4 g daily.

Children:

Antibacterial:

Children 8 years of age and over: 6.25 to 12.5 mg per kg of body weight every 6 hours; or 12.5 to 25 mg per kg of body weight every 12 hours.

- Patient should continue medicine for full course of treatment.
- Do not take within 1 to 3 hours of other medicine, milk or other dairy products.

CONTRAINDICATIONS:

Tetracycline should not be used in children below 12 years as it causes tooth discolouration. It is also contraindicated in patients hypersensitive to tetracyclines and patients with renal impairment.

WARNING AND PRECAUTIONS:

It should be used with caution in patients with renal function impairment and hepatic dysfunction. Absorption from oral dose is impaired by food and milk, antacids, mineral supplements such as iron, calcium or magnesium when taken concomitantly.

INTERACTIONS WITH OTHER MEDICAMENTS:

Combinations containing any of the following medications may interact with this medication: antacids, calcium supplements, iron supplements, magnesium-containing laxatives, contraceptives, penicillins and barbiturates.

PREGNANCY AND LACTATION:

Tetracycline should not be used in the latter half of pregnancy.
It is also contraindicated during lactation in women breast feeding infants.

SIDE EFFECTS:

Discolouration of infants' or children's teeth. Cramps or burning of the stomach. Darkened or discoloured tongue due to fungal overgrowth. Hypertrophy of the papilla. Diarrhoea. Increased sensitivity to sunlight. Itching of the rectal or genital areas or sore mouth or tongue due to fungal overgrowth. Gastrointestinal disturbances including nausea and vomiting may occur.

Skin: Maculopapular and erythematous rashes. Exfoliative dermatitis has been reported. Onycholysis and discolouration of the nails and photosensitivity.

Hypersensitivity reactions: Urticaria, angioneurotic edema, anaphylaxis, anaphylactoid purpura, pericarditis, exacerbation of systematic lupus erythematosus and serum sickness-like reactions, as fever, rash and arthralgia.

Blood: Anemia, haemolytic anemia, thrombocytopenia, thrombocytopenic purpura, neutropenia and eosinophilia have been reported.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Overdosage may cause gastrointestinal tract (GIT) disturbances like nausea, vomiting, and diarrhoea. Overdosage can cause fatal liver damage. It is believed that blood levels for tetracycline that exceed 16 mcg/ml of serum contribute to the development of liver damage.

Treatment: Discontinue drug. Treat GIT disturbances by giving milk every 3 hours alternating with bismuth subcarbonate, 5 g every 3 hours.

PACKING/PACK SIZE(S):

Blister-packs of 50x10's and 100x10's.

**JAUHI UBAT DARIPADA KANAK-KANAK
KEEP OUT OF REACH OF CHILDREN**

MANUFACTURER/PRODUCT REGISTRATION HOLDER:

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